Emergency Codes, Disaster Management, Patient Safety and Data Privacy

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1. Purpose

This document lists the emergency codes, the disaster and fire procedures, the patient safety rules and the data privacy and AI use rules at CareOps General Hospital.

2. Emergency codes

The hospital uses colour codes announced by the hospital operator. Code Blue means a cardiac or respiratory arrest and the resuscitation team must arrive within 3 minutes. Code Red means fire. Code Yellow means a mass casualty incident or external disaster. Code Pink means a missing or abducted infant or child. Code Black means a bomb threat. Code Grey means a violent or threatening person and security responds. Code Orange means a hazardous material or chemical spill. These codes are internal hospital policy, and other hospitals may use different colours. The operator announces the code, the location, and the all clear.

3. Fire safety

Fire response follows the RACE steps. R is Rescue anyone in immediate danger. A is Alarm, which means activating the nearest fire alarm and calling the operator. C is Confine, which means closing doors and windows to limit the spread of fire and smoke. E is Extinguish if the fire is small and it is safe, or Evacuate. A portable extinguisher is used with the PASS steps: Pull the pin, Aim at the base of the fire, Squeeze the handle and Sweep from side to side. Lifts must not be used during a fire. Patients are evacuated horizontally to a safe compartment on the same floor first, then vertically by stairs if needed. The fire emergency number in India is 101.

4. Mass casualty incident

When a mass casualty incident is declared with Code Yellow, the Medical Superintendent activates the hospital incident command system. The ED is cleared of stable patients, who are moved to wards or discharged when safe. Elective work stops. Surge beds are opened. Patients are sorted using the START method, which means Simple Triage and Rapid Treatment. START assigns four categories: Red for immediate, Yellow for delayed, Green for minor, and Black for deceased or not expected to survive. The sorting is based on the ability to walk, breathing, circulation and mental status. A triage officer, a treatment area leader, a logistics officer, a security officer and a communications officer are named. A family reception area and a media point are set up away from the ED. Relevant public numbers in India are 112 for national emergency, 100 for police, 101 for fire and 102 or 108 for ambulance.

5. Patient safety

Every patient is identified using at least two identifiers, for example full name and date of birth or hospital number, before medicines, blood, specimens or procedures. This is one of the international patient safety goals. Medicines follow the five rights: right patient, right drug, right

dose, right route and right time. Falls risk assessment is done on admission. Pressure injury risk is assessed on admission. Staff report all incidents and near misses in the incident reporting system within 24 hours. A sentinel event is an unexpected event that causes death or serious harm and it is reported to the Medical Superintendent immediately and reviewed with a root cause analysis within 45 days.

6. Quality standards

In India, hospitals can seek accreditation from the National Accreditation Board for Hospitals and Healthcare Providers, called NABH, which is a constituent board of the Quality Council of India. Internationally, Joint Commission International accredits hospitals. CareOps General Hospital uses these standards as a reference for quality improvement. The Quality Office reports operational indicators such as waiting time, occupancy, infections and incidents to the hospital leadership every month.

7. Data privacy

Patient information is confidential. Staff access to records is limited to what is needed for their role. In India, the Digital Personal Data Protection Act 2023 governs the processing of digital personal data. In the United States the HIPAA Privacy Rule protects health information. The CareOps AI prototype uses only synthetic or anonymised data and must not store or display real patient names, phone numbers, addresses or identification numbers.

8. Rules for the CareOps AI assistant

The assistant supports hospital administrators with operations questions about beds, waiting times, workload, staffing, equipment and approved procedures. It must answer only from the hospital database and the approved documents. It must say clearly when the information is not available. It must not give medical diagnosis, treatment or medicine advice, and must tell the user to consult a qualified clinician for medical questions. Its recommendations are decision support, and a human manager makes the final decision. It must name the document or data source used when it quotes a policy or a number.